

Unmada and Apasmara

Learning Objectives

- ⇒ To understand the concept of Unmada and Apasmara
- ⇒ To know the Nirukti of Unmada and Apasmara
- ⇒ To understand the Nidana and Samprapti of Unmada mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of the disease
- ⇒ To enlist the Bheda (types) of Unmada and Apasmara
- ⇒ To know the Samanya Lakshana of Unmada and Apasmara

Introduction

This is a disease of *Manovaha Srotas* where the person will have abnormal functioning of the Satva or *Manas* (mind). In this disease, along with *Sharirika Dosha* involvement, there will be the affliction of *Rajas Dosha* is involved. The involvement of *Rajas Dosha* implies the unsteadiness of the clinical symptoms of *Unmada*. Whenever, we speak of psychological activities, first of all it starts with a mindset which leads to a particular thought process which leads to a peculiar feeling pertaining to that particular thought process which in turn leads to a specific type of action. If the individual is repeating similar type of action, that will lead to a particular result which in turn determines one's destiny. If any individual wants to work on development of one's destiny, he should work on training one's mindset. If any of the above said parameters are not met, then the person may show abnormal behaviours. If the person is suffering from *Unmada*, the abnormal pattern of activities can be observed either in mindset or in thought or in feeling or in action or in result which in turn alters

one's destiny. In the coming pages, we are going to discuss the extent of meaning of *Unmada*.

Nirukti of Unmada

मदयन्त्युद्धता दोषा यस्मादुन्मार्गमाश्रिताः।

मानसोऽयमतो व्याधिरुन्माद इति कीर्तितः॥३॥

Su.Ut. – 62/3

- ⊗ When aggravated *Dosha* takes upward movement and take shelter in *Manovaha Srotas* to cause disturbance in the *Manas*. That is the reason, this disease is termed as *Unmada*.

Pratyatma Lakshana (cardinal feature)

उन्मादं पुनर्मनोबुद्धिसंज्ञाज्ञानस्मृतिभक्तिशील
विद्यात्॥५॥

चेष्टाचारविभ्रमं

Ca.Ni. – 7/5

Pratyatma Lakshana of *Unmada* include

- ⊗ **Mano Vibhrama¹** – perversion of mind
- ⊗ **Buddhi Vibhrama²** – perversion of intellect

Ca.Ni. – 7/5, Chakra

1 अत्र मनोविभ्रमात् चिन्तयन्तर्थात्र चिन्तयते, अचिन्त्यांश्च चिन्तयते।

When an individual is supposed to think in a particular situation, if he is not thinking in that particular fashion and if an individual is performing the activities as per his wish can be considered as *Mano Vibhrama*. For example, the farmer avoiding the thought processes related to ploughing or crop preserving the activities and if he is unable to do the activities as per his wish can be considered as *Mano Vibhrama*. For example, the farmer by profession, if he is thinking in terms of construction of different houses avoiding the thought processes related to ploughing or crop preserving the activities and if he is unable to do the activities as per his wish can be considered as *Mano Vibhrama*. For example, the farmer by profession, if he is thinking in terms of construction of different houses avoiding the thought processes related to ploughing or crop preserving the activities and if he is unable to do the activities as per his wish can be considered as *Mano Vibhrama*.

2 बुद्धिविभ्रमानु नित्यमनित्यमिति. प्रियं चाप्रियमिति पश्यति. विषयाभिनिवेशो यो नित्यानित्ये प्रियाप्रिये. ज्ञेयः स बुद्धिविभ्रमः, Ca.Ni. – 7/5, Chakra

When an individual is suffering from *Buddhi Vibhramsha*, he is unable to identify which is good or bad or which is pleasing or unpleasing. For example, an individual is holding the knife and if he is unable to identify its use, then such type of state is termed as *Buddhi Vibhramsha*.

- **Samjna Vibhrama** – perversion of consciousness
- **Jnana Vibhrama**³ – perversion of knowledge
- **Smriti Vibhrama**⁴ – perversion of memory
- **Bhakti Vibhrama**⁵ – perversion of desire
- **Shila Vibhrama**⁶ – perversion of manner or patterns
- **Cheshta Vibhrama**⁷ – perversion of behaviour
- **Achara Vibhrama**⁸ – perversion of conduct

Nidana of Unmada

विरुद्धदुष्टाशुचिभोजनानि प्रधर्षणं देवगुरुद्विजानाम्।

उन्मादहेतुर्भयहर्षपूर्वी मनोऽभिघातो विषमाश्च चेष्टाः॥4॥ Ca.Ci. – 9/4

- **Viruddha Bhojana** – consumption of incompatible food substances
- **Dushta Bhojana** – consumption of spoilt food substances
- **Ashuchi Bhojana**⁹ – consumption of unclean food substances

- **Deva, Guru, Dvija Praghharshana**¹⁰ – showing disrespect to heavenly bodies or Gods, preceptors or teachers or twice born
- **Mano Abhighata due to Bhaya**¹¹ – affliction of mind due to sudden emotion like fear
- **Mano Abhighata due to Harsha**¹² – affliction of mind due to sudden joy
- **Mano Abhighata due to Vishama Cheshta**¹³ – affliction of mind due to violent physical activities

Samprapti of Unmada

तैरल्पसत्त्वस्य मलाः प्रदुष्टा बुद्धेर्निवासं हृदयं प्रदूष्य।

स्त्रोतांस्यधिष्ठाय मनोवहानि प्रमोहयन्त्याशु नरस्य चेतः॥5॥

Ca.Ci. – 9/5

- When the people are having poor mental strength, there is vitiation of *Dosha*
- Vitiated *Dosha* get invade the *Hridaya*, the seat of *Manas*, get lodged in all the channels of *Manas*

A person who is holding the pen, in stead of using it for writing, if he uses for purposes other than writing, then that may suggest *Buddhi Vibhrama*. Ca.Ni. – 7/5, Chakra

- 3 संज्ञां ज्ञानं, तद्विभ्रमात् अग्न्यादिदाहं न बुद्ध्यते, किंवा संज्ञां नामोल्लेखेन ज्ञानम्।
When an individual is having *Samjna* and *Jnana Vibhrama*, then he is not in position to perceive the burning sensation which develops when he reaches near the fire or the person is unable to name *Agni* (fire). After seeing the vegetables, he is not in a position to identify the vegetable or unable to say its usage. Ca.Ni. – 7/5, Chakra

- 4 स्मृतिविभ्रमात् न स्मरति, अयथावद्वा स्मरति।
In this situation, the person is unable to remember what he has taken in the breakfast or he will have inability to remember the names of the near relatives or he will have inability to remember the date of birth or anniversary or he may have inability to perceive the smell of the rose etc. Ca.Ni. – 7/5, Chakra

- 5 भक्तिरिच्छा, तद्विभ्रमाच्च यत्रेच्छा पूर्वमासीत्तत्रानिच्छा भवति।
The term *Bhakti* refers to desire or liking. For example, if an individual is very fond of children, in due course of time, if he dislikes children. If a woman is very fond of friendship, in due course of time, if he prefers to sit alone avoiding friends could be considered in the context of *Bhakti Vibhrama*. Ca.Ni. – 7/5, Chakra

- 6 शीलविभ्रमादक्रोधनः क्रोधनो भवति।
The term *Shila* refers to repetitive activities or following a particular pattern. Textual example says if a person who is not getting anger and if he frequently getting irritated, then it can be considered as *Shila Vibhrama*. Further, if an individual frequently sits alone in crowded places without doing any activities can be another example. Ca.Ni. – 7/5, Chakra

- 7 चेष्टाविभ्रमादनुचित चेष्टो भवति।
When an individual is getting scolding from a total stranger, getting anger could be a normal phenomenon. In stead, if he gets smile, it can be considered as *Cheshta Vibhrama*. If an individual is losing his dear ones, then sorrow is one of the natural reaction or response. In stead, if he starts laughing, then it can be considered as *Cheshta Vibhrama*. Ca.Ni. – 7/5, Chakra

- 8 आचारः शास्त्रशिक्षाकृतो व्यवहारः, तद्विभ्रमादशीचाद्याचरति।
The term *Achara* refers to the activities or skills mentioned in the textbooks or learnt during learning days. If a farmer by profession, if he plants the seeds during the summer season or if an individual is computer engineer by profession, if he finds difficulty in operating in the computer can be the examples for *Achara Vibhrama*. Ca.Ni. – 7/5, Chakra

- 9 All these aetiological factors need not cause disease manifestation in a single setting. It requires longer exposure to these aetiological factors. All these aetiological factors will lead to inflammation of the gastric and/ or intestinal mucosa. This will lead to poor nutrient absorption in terms of glucose, sodium, potassium, calcium ions. Disorientation or poor orientation of space and time can be considered as transient until an unless the person receives adequate electrolyte supplementation.

- 10 Deva, Guru or Dvija are usually considered as mentors. When a person strongly believes mentors and follows what is said from the mentors. That knowledge helps oneself to move ahead in one's life very rapidly. If the people do not believe their mentors, then either person may have the illusion regarding his venture success.

- 11 When an individual is repeatedly exposed to fearful situation or situation where he is exposed to continuous abusive language, then he may likely to go into a state of depression.

- 12 When the child is pampered excessively in a single child family or wealthy family and he gets what he desired immediately, this may lead to a state of elated mood. He may start with abnormal tantrums when he is denied any of the desired things.

- 13 When an individual is frequently attaining abnormal physical postures either due to self or attained by others for a longer period, then such a phenomenon may lead to affliction of psyche of the individual.

- This mechanism will bring about the derangement quickly

Samprapti Ghataka

Dosha : Rajas dominant Sharirika Dosha

Dushya : Rasa, Manas

Srotas : Rasavaha, Manovaha Srotas

Srotodushti Prakara : Sanga and Vimargagamana

Udbhava Sthana : Amashaya

Sanchara Sthana : Sarva Sharira

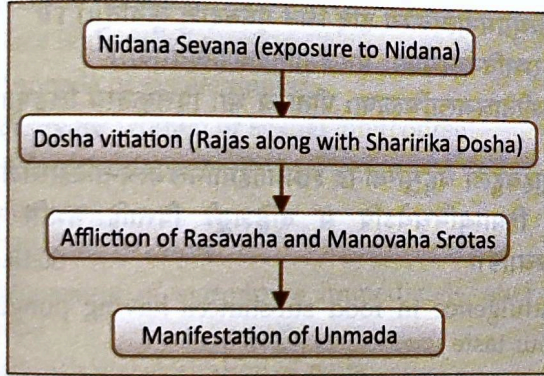
Vyakta Sthana : Hridaya (due to abnormal functioning of Manas)

Vyadhi Prakara : Manasika

Vyadhi Svabhava : Ashukari to Chirakari

Sadhyasadyata : Sadhya to Pratyakhyeya

Flowchart of Samprapti of the disease



Purvarupa of Unmada

मोहोद्वेगौ स्वनः श्रोत्रे गात्राणामपकर्षणम्।

अत्युत्साहोऽरुचिश्चाने स्वप्ने कलुषभोजनम्॥6॥

वायुनोन्मथनं चापि भ्रमश्च क्रमतस्तथा।

यस्य स्यादचिरेणैव उन्मादं सोऽधिगच्छति॥7॥

Su.Ut. - 62/6-7

तस्येमानि पूर्वरूपाणि तद्वथाशिरसः शून्यता, चक्षुषोरकुलता, स्वनः कर्णयोः, उच्छ्वासस्याधिक्यम्, आस्यसंस्ववणम्, अनन्नाभिलाषारोचकाविपाकाः, हृद्रहः, ध्यानायाससंमोहोद्वेगाश्चास्थाने, सततं लोमहर्षः, ज्वरश्चाभीक्षणम्, उन्मत्तचित्तत्वम्, - दर्दित्वम्, अर्दिताकृतिकरणं च व्याधेः, स्वप्ने चाभीक्षणं दर्शनं भ्रान्तचलितानवस्थितानां रूपाणामग्रश-स्तानां च तिलपीडकचक्राधिरोहणं वातकुण्डलिका- भिश्चन्मथनं निमज्जनं च कलुषाणामम्भसामावर्तं चक्षुषोश्चापसर्पणमिति (दोषनिमित्तानामुन्मादानां पूर्वरूपाणि भवन्ति)॥6॥

Ca.Ni. - 7/6

- **Moha**¹⁴ – altered state of consciousness
- **Udvega**¹⁵ – excitement
- **Karna Svana**¹⁶ – ringing in the ears
- **Gatra Apakarshana**¹⁷ – weakness of the body parts
- **Atyutsaha**¹⁸ – excess enthusiasm
- **Aruchi** – tastelessness
- **Svapne Kalusha Bhojana**¹⁹ – consumption of unclean food in dreams
- **Vayuna Unmathana**²⁰ – anxious look of the patient
- **Bhrama**²¹ – giddiness

Samanya Lakshana of Unmada²²

धीविभ्रमः सत्त्वपरिप्लवश्च पर्याकुला दृष्टिरधीरता च।

अवद्ववात्तत्वं हृदयं च शून्यं सामान्यमुन्मादस्य लिङ्गम्॥ 6॥

Ca.Ci. - 9/6

- **Dhi Vibhrama**²³ – improper understanding
- **Satva Pariplava**²⁴ – unsteadiness of mind

- 14 This may be developed when the person is presented with head injury or cerebro vascular accidents or in brain tumours or in abnormalities of reticular activating system. This may be seen in electrolyte imbalances too. These conditions may affect mental function of the individual including memory or orientation of space and time or hallucinations or delusions and abnormal conduct.
- 15 This is attributable to unstable mood elevation and he may show aggression or enlightenment even for a trivial reason.
- 16 This can be attributable to auditory hallucinations. This can be considered as one of the features of schizophrenia.
- 17 These Lakshana may appear when the individual is having poor nutrients in the body such as hyponatremia or hypokalemia or hypoglycemia or hypovolemia. This may lead to poor mental function.
- 18 This can be attributable to elated mood or mood elevation where the person shows enthusiasm in beginning the activities.
- 19 This Lakshana is difficult to interpret.
- 20 This is attributable to anxiety state of the individual. This can be identified by frequent occurrence of pursed lip breathing.
- 21 Hyperventilation is the most probable cause of giddiness. This phenomenon is observed when he is having excessive anxiety.
- 22 T Harv Eker says a formula TFAR. Where T refers to Thought; F refers to Feeling; A refers to Action and R refers to Result. For any action to occur, there should be a thought process to be developed in an individual. This thought process should have certain emotion so that appropriate action can be taken which may lead to certain kind of result. If one wants to have desired result, he should have that thought process which helps oneself to attain the desired result. If we develop the same kind of result again and again decides one's destiny.
- 23 We can modify this formula into MTFARD where M refers to Mindset and D refers to Destiny. The desired mindset can be achieved by controlling one's This formula implies one's destiny is determined by the nature of one's mindset. If any one or two or more of MTFARD is not aligned with one's core subconscious mind which is 9 times more powerful than conscious mind. If any one or two or more of MTFARD is not aligned with one's core values, leads to the abnormal behavioural patterns which may be the features of Unmada.
- 24 The person is unable to perceive the good and bad or true and false or right and wrong etc. In this, the person may have disorientation of space and time, memory disturbances and abnormal behaviour patterns.
- 25 This is attributable to instability of mind. In other words, he cannot judge what is right and wrong for him. His decisions are influenced by

- **Paryakula Drishti**²⁵ – In coordination of sight
- **Adhirata**²⁶ – Feeling of fear
- **Abaddha Vaktva**²⁷ – Irrelevant and incomprehensible speech
- **Shunya Hridaya**²⁸ – Emptiness of heart or mind

Sankhya Samprapti of Unmada

5 types:

1. Vataja Unmada
2. Pittaja Unmada
3. Kaphaja Unmada
4. Sannipataja Unmada
5. Agantuja Unmada

As per Sushruta, 6 types of Unmada are mentioned

1. Vataja Unmada
2. Pittaja Unmada
3. Kaphaja Unmada
4. Sannipataja Unmada
5. Manasa Unmada
6. Vishaja Unmada

Vataja Unmada

रुक्षाल्पशीतान्नविरेकधातुक्षयोपवासैरनिलोऽतिवृद्धः।
चिन्तादिजुष्टं हृदयं प्रदूष्य बुद्धिं स्मृतिं चाप्युपहन्ति शीघ्रम्॥१॥
अस्थानहासस्मितनृत्यगीत वागङ्गविक्षेपणरोदनानि।
पारुष्यकार्यारुणवर्णताश्चजीर्णे बलं चानिलजस्य रूपम्॥१०॥
Ca.Ci. – 9/8-10
रुक्षच्छविः परुषवाग्धमनीततो वा श्वासातुरः कृशतनुः स्फुरिताङ्गसन्धिः।
आस्फोटयन्नटति गायति नृत्यशीलो विक्रोशति भ्रमति चाप्यनिल-
प्रकोपात्॥८॥
Su.Ut. – 62/8

- Indulgence in food substances which are dry, scanty, cold
- Frequent purificatory procedure i.e. emesis and purgation
- Depletion of body tissues
- Fasting or starvation and other factors which vitiate Vata Dosha
- The psychological triggers such as worry etc in a person having weak Manas will bring about derangement of intellect and memory.

Clinical features

Clinical features of Vataja Unmada are as follow:

- Laughing at improper time and place
- Smiling at improper time and place
- Speaking at improper time and place
- Singing at improper time and place
- Dancing at improper time and place
- Making body movements at improper time and place
- Hardness or roughness of body parts
- Emaciation of the body
- Blackish red discolouration of the body
- Exacerbation of the symptoms after the digestion of food

Pittaja Unmada Lakshana

अजीर्णकल विदाहशीतै- भौज्यैश्चितं पित्तमुदीर्णवेगम्।
उन्मादमत्युग्रमनात्मकस्य हृदि श्रितं पूर्ववदाशु कुर्यात्॥११॥
अमर्षसंरम्भविनग्नभावाः संतर्जनातिद्रवणौष्ण्यरोषाः।
प्रच्छायशीतान्नजलाभिलाषाः पीता च भाः पित्तकृतस्य लिङ्गम्॥१२॥
Ca.Ci. – 9/11-12

तृस्वेददाहबहुलो बहुभुग्विनिद्र श्छायाहिमानिलजलान्तविहारसेवी।
तीक्ष्णो हिमाम्बुनिचयेऽपि स वह्निशङ्की पित्ताहि नभसि पश्यति
तारकाश्च॥९॥
Su.Ut. – 62/9

- Indulgence in food substances having pungent or sour taste
- Indulgence in food which cause heartburn
- Indulgence in food substances which are hot in nature
- This will lead to the vitiation of Pitta quickly, which the people who are not self controlled

Clinical features

Clinical features of Pittaja Unmada are

- Intolerance
- Uncontrollability
- Casting away the clothes and remaining naked
- Threatening others
- Running away
- Feeling of burning sensation
- Desiring shade, cold water and foods
- Yellowish discolouration of the body

- 25 This is attributable to unsteady vision. In other words, his attention is varied and he cannot fix his attention to one particular object or thought process. Even his attention cannot be drawn by other individuals.
- 26 This is attributable to lack of initiation to do any of the activities. He may have fear of various kinds (fear of rejection or fear of failure etc).
- 27 This is attributable to either increased or decreased speech. It can be attributable to either non fluent or fluent speech. Non fluent speech indicates the involvement of motor area involvement of brain and fluent speech indicates the affliction of sensory area involvement. As per current understanding, speech faculty includes reading, writing, listening and speaking. In this situation, speaking part has to be considered.
- 28 This can be attributable to the state of emotionlessness.

to *Smarana* (*Smriti*) or memory. In this disease, the person will be presented with loss of memory for a transient period of time. This is associated with vitiation of *Sharirika Dosha* along with involvement of *Tamo Dosha*. The clinical features mentioned in *Apasmara* are comparable to seizure attacks. A seizure is defined as the paroxysmal event due to abnormal, excessive, hypersynchronous discharges from an aggregate of central nervous system (CNS) neurons.

Nirukti

स्मृतिर्भूतार्थविज्ञानमपश्च परिवर्जने।

अपस्मार इति प्रोक्तस्ततोऽयं व्याधिरन्तकृत्॥३॥

Su.Ut. – 61/3

As per *Sushruta*, in this disease, the person will lose the experience of past and knowledge of nature of things and kills the ailing person. In this definition, the term *Smriti* refers to the recollection of past events or objects. In this set of diseases, the person will be presented with loss of memory and this may lead to death. In other way, it can be explained as the one who lost the experience of past and knowledge of nature of things is known as *Apasmara* and it will kill an individual.

स्मृतेरपगमं प्राहुरपसारं भिषग्विदः।

तमः प्रवेशं बीभत्सचेष्टं धीसत्त्वसंलवात्॥३॥

Ca.Ci. – 10/3

अपस्मारं पुनः स्मृतिबुद्धिसत्त्वसंलवाद्बीभत्सचेमावस्थिकं तमः प्रवेशमाचक्षते॥५॥

Ca.Ni. – 8/5

As per *Caraka*, *Apasmara* is comprises of 3 major cardinal features.

- **Smriti, Buddhi and Satva Samprava** – there will be loss of memory, intellect
- **Bhibhatsa Cheshta** – appearance of froth through mouth and involuntary movements in the body
- **Avasthika Tamapravesha** – transient loss of consciousness
- **Vegavan (Chakrapani commentary)** – the symptoms appearing in episodes of above said clinical features

Nidana of Apasmara

विभ्रान्तबहुदोषाणामहिताशुचिभोजनात्।

रजस्तमोभ्यां विहते सत्त्वे दोषावृते हृदि॥४॥

चिन्ताकामभयक्रोधशोकोद्वेगादिभिस्तथा।

मनस्यभिहते नृणामपस्मारः प्रवर्तते॥५॥

Ca.Ci. – 10/4-5

- **Vibhranta Bahudosh³¹** – *Dosha* aggravated by faulty and excessive use of sense organs
- **Ahita and Ashuchi Bhojana³²** – excess consumption of incompatible and contaminated food substances
- **Rajastamobhyam Vihata Satva³³** – mind overpowered by *Manasa Dosha* i.e. *Rajas* and *Tamas*
- **Chinta, Kama, Bhaya, Krodha, Shoka and Udvega³⁴** – excess exposure to worry, lust, fear, anger, grief and anxiety

मिथ्यातियोगीन्द्रियार्थकर्मणामभिसेवनात्।

विरुद्धमलिनाहारविहारकुपितैर्मलैः॥४॥

वेगनिग्रहशीलानामहिताशुचिभोजनात्।

2. Brain tumour that compresses the area of the brain
3. Destroyed area of brain tissue
4. Congenitally deranged local circuitry

31 If an individual is indulged in activities which lead to fear such as seeing or hearing crime stories or psychothriller stories etc frequently, that will create emotional lability which in turn activates amygdala. Amygdala is a structure which comprises of multiple small nuclei complex located immediately beneath the cerebral cortex of the medial anterior pole of each temporal lobe. This receives neuronal signals from all portions of limbic cortex as well as neocortex of the temporal, parietal and occipital lobes, especially from auditory and visual association areas. Effects initiated from amygdala will be sent through hypothalamus include

1. Increase or decrease in heart rate
2. Increase or decrease in arterial pressure
3. Increase or decrease in gastrointestinal motility and secretion
4. Defaecation or micturition
5. Pupillary dilatation or rarely constriction
6. Piloerection
7. Secretion of various anterior pituitary hormones

Apart from above said effects mediated through hypothalamus, it can also cause certain types of involuntary movement.

1. Tonic movements, such as raising the head or bending the body
2. Circling movements
3. Occasional clonic, rhythmic movements
4. Different types of movements associated with olfaction and eating, such as licking, chewing and swallowing.

32 When an individual is consuming incompatible and contaminated food substances, the immediate response could be the person may develop either vomiting or diarrhoea associated with gastrointestinal symptoms manifestation. This will lead to abrupt nutrient loss, carbohydrates in particular which leads to the excess formation of ketoacidosis and lactic acidosis. This state is called as metabolic acidosis. This may lead to the state of respiratory alkalosis. Respiratory alkalosis is one of the predisposing factor for the development of seizure attacks. S.N. : Respiratory alkalosis is caused by either hypoxia or metabolic acidosis or increased metabolic demand such as fever etc.

33 *Rajas* and *Tamas* are considered as two *Manodoshas* which are responsible for multiple *Manasika Vikara*. Anger, irritability etc will cause the vitiation of *Rajo Dosha* and sorrow, grief etc will cause the vitiation of *Tamo Dosha*. This may lead to emotional lability or extreme emotional disturbances and these may lead to the precipitation of epileptic attacks.

34 These are extreme emotional disturbances which may induce episodes of epilepsy.

Kaphaja Unmada Lakshana

संपूर्णैर्मन्दविचेष्टितस्य छर्दिश्च लाला च बलं च भुङ्क्ते सोष्मा कफो
मर्मणि संप्रवृद्धः।

बुद्धिं स्मृतिं चाप्युपहत्य चित्तं प्रमोहयन् संजनयेद्विकारम्॥1 3॥

वाक्चेष्टितं मन्दमरोचकश्च नारीविविक्तप्रियताऽतिनिद्रा।

नखादिशौक्ल्यं च कफात्मकस्य॥1 4॥

Ca.Ci. – 9/13-14

छर्दिग्निसादसदनारुचिकासयुक्तो योषिद्विविक्तरतिरल्पमतिप्रचारः।

निद्रापरोऽल्पकथनोऽल्पभुगुष्णासेवी रात्रौ भृशं भवति चापि
कफप्रकोपात्॥1 0॥

Su.Ut. – 62/10

- Indulgence in food intake- over eating
- Lack of physical exercise
- Derangement of the intellect and memory leading to Kaphaja Unmada

Clinical features

Clinical features of Kaphaja Unmada are

- Weak or slow voice
- Weak or slow talk
- Slow body movements
- Anorexia or tastelessness
- Desire for women or sexual intercourse
- Excess sleep
- Vomiting
- Dribbling of saliva
- Worsening of symptoms immediately after taking food
- Whitish discolouration of nails

Sannipataja Unmada

यः सन्निपातप्रभवोऽतिघोरः सर्वैः समस्तैः स च हेतुभिः स्यात्।

सर्वाणि रूपाणि बिभर्ति ताहविरुद्धं भ्रैषज्यविधिर्विवर्ज्यः॥1 5॥

Ca.Ci. – 9/15

सर्वात्मके पवनपित्तकफा यथास्वं संहर्षिता इव च लिङ्गमुदीरयन्ति॥1 1॥

Su.Ut. – 62/10

29 To understand the exact meaning of this type of Unmada, one has to have a thorough knowledge of Manasa types of Prakriti and subtypes of Manasa Prakriti, Sathvika, Rajasika and Tamasika Prakriti.

30 This disease is associated with loss of consciousness and memory for a considerable period of time. As far as Pratyatma Lakshana of Apasmara is concerned, most of the features mentioned in Apasmara mimics the features of grand mal epilepsy. The factors which increase the excitability of abnormal epileptogenic circuitry enough to induce the seizures or epileptic attacks include

1. Strong emotional stimuli
2. Alkalosis caused by overbreathing
3. Drugs
4. Fever
5. Loud noises or flashing lights

Petit mal epilepsy is a minor form of epilepsy involves thalamocortical brain activating system. It is usually characterised by 3 to 30 seconds of unconsciousness, during which the person may have twitch like contractions of the muscles in the lead region, particularly blinking of the eyes and it is followed by return of consciousness and resumption of physical activities. The patient may have one such attack in many months or in rare instances, may have rapid series of attacks one after the other. The usual course for petit mal epilepsy to appear first during late childhood and then to disappear by the age of 30 years. On occasion, a petit mal epileptic attack can predispose the grand mal epilepsy.

Focal epilepsy can involve almost any local part of the brain, either localised regions of the cerebral cortex or deeper structures of both cerebrum and brain stem. The probable causes of focal epilepsy are as follow:

1. Scar tissue in the brain stem that pulls on the adjacent neuronal tissue

• This type of Unmada is manifested by the vitiation of all three Dosha by the vitiation of Dosha by respective causes.

• The clinical features of all the Dosha are seen in the patient and requires treatment of different kinds.

• This condition should be rejected to be treated by the physicians or clinicians.

Agantuja Unmada

देवर्षिगन्धर्वपिशाचयक्षरक्षः पितृणामभिधर्षणानि।

आगन्तुहेतुर्नियमत्रतादिमिथ्याकृतं कर्म च पूर्वदेहे॥1 6॥ Ca.Ci. – 9/16

• The term Agantuja Unmada²⁹ refers to the manifestation of Unmada due to external causes. Due to the curse of Deva, Rshi, Gandharva, Pischacha, Yaksha, Raksha, Pitr are responsible for the manifestation of Unmada.

त्रिविधं तु खलून्मादकराणां भूतानामुन्मादने प्रयोजनं भवति;
तद्यथा - हिंसा, रतिः, अभ्यर्चनं चेति॥ Ca.Ni. – 7/15

Himsa, Rati and Abhyarchana are three forms of reactions observed in Agantuja Unmada. Abhyarchana type of reaction is seen in Satva Pradhana Unmada such as Pitr Unmada or Deva Unmada. In this type of reflection, the suffering individual will show continuous, excessive devotion or worshipping to one particular diety or object. Rati type of reaction is seen in Vataja or Rajo dominant Unmada such as Gandharva or Yaksha Unmada. In this type of reflection, we can find excess, uncontrollable desire to one particular object. Sometimes, this type of behaviour is irritating to the individuals too. Himsa type of reaction is seen in Kapha or Tamo dominant Unmada such as Raksha or Rakshasa or Pischacha Unmada. This type of reflection is mainly identified by the attainment of pleasure by causing harm to other individuals.

Introduction

Apasmara³⁰ comprises of two words – Apa and Smara. Here, the term Apa refers to loss or elimination and Smara refers

रजस्तमोभिभूतानां गच्छतां च रजखलाम्॥५॥
तथा कामभयोद्वेगक्रोधशोकादिभिर्भूशम्।
चेतस्युपहते पुंसामपस्मारोऽभिजायते॥६॥

Su.Ut. – 61/4-6

- **Mithyatiyoga of Indriyārtha and Karma³⁵** – improper or excessive use of sense organs and physical, mental and verbal activities
- **Viruddha Ahara and Vihara** – consumption of incompatible food and activities
- **Malina Ahara and Vihara³⁶** – consumption of unclean food and activities
- **Vega Nigraha³⁷** – suppression of natural urges
- **Ahita Bhojana** – consumption of unhealthy food substances
- **Ashuchi Bhojana³⁸** – consumption of contaminated food substances
- **Raja Tamo Abhibhutanam** – affliction of mind with *Raja* and *Tamo Dosha*
- **Rajasvalam Cha Gacchatam³⁹** – sexual intercourse with women during menstruation
- **Kama, Bhaya, Udvega, Krodha and Shoka etc⁴⁰** – during the state of lust, fear, excitement, anger and grief etc

त एवंविधानां प्राणभृतां क्षिप्रमभिनिर्वर्तन्ते; तद्यथा -
रजस्तमोभ्यामुपहतचेतसामुद्रान्तविषम- दोषाणां समल-
विकृतोपहितान्यशुचीन्यव्यवहार- जातानि वैपम्ययुक्ते-नोपयो
गविधिनोपयुञ्जानानां तन्त्रप्रयोगमपि च विषममाचरतामन्याश्च
शरीर- चेष्टा विषमाः समाचरतामत्युपक्षयादा दोषाः प्रकु-
पिता रजस्तमोभ्यामुपहतचेतसामन्तरात्मनः श्रेष्ठ- तममायतनं
हृदयमुपसृत्योपरि तिष्ठन्ते तथेन्द्रिया- यतनानि च।
तत्र चावस्थिताः सन्तो यदा हृदय- मिन्द्रियायतनानि चेरिताः
कामक्रोधभयलोभमोह- हर्षशोकचिन्तोद्वेगादिभिः सहसाऽभिपूरयन्ति,
तदा जनुरपस्मरति॥४॥

Ca.Ni. – 8/4

- The people whose mind is afflicted by vitiated *Rajas* and *Tamas Dosha*

- Who have scattered, abnormal and plenty of *Dosha*
- The people who don't follow application of rules and regulations of diet
- The people who don't follow *Tantric* practices properly
- Abnormal body postures
- Excess depletion of body tissues
- Due to above said reasons, the *Dosha* get vitiated and *Rajas* and *Tamas Dosha* get vitiated and spread over *Hridaya*, the best seat of inner self and also to other seats of sense organs and stay there predominantly while residing there when they are excited by passion, anger, fear, greed, confusion, exhilaration, grief, anxiety, agitation etc and fill up *Hridaya* and seats of sense organ as a result the person gets bouts of *Apasmara*.

Samprapti

धमनीभिः श्रिता दोषा हृदयं पीडयन्ति हि।

संपीड्यमानो व्यथते मूढो भ्रान्तेन चेतसा॥६॥

Ca.Ci. – 10/6

संज्ञावहेषु स्रोतःसु दोषव्याप्तेषु मानवः।

रजस्तमः परीतेषु मूढो भ्रान्तेन चेतसा॥८॥

विक्षिपन् हस्तपादौ च विजिह्वभ्रूलोचनः।

दन्तान् खादन् वमन् फेनं विवृताक्षः पतेत् क्षितौ॥९॥

अल्पकालान्तरं चापि पुनः संज्ञां लभेत सः।

सोऽपस्मार इति प्रोक्तः

Su.Ut. – 61/8-10

- Due to exposure to etiological factors, *Dosha* (*Sharirika* as well as *Manasika*) become vitiated, *Tamo Dosha* in particular
- These vitiated *Dosha* reach *Samjnavaha Srotas* via *Dhamani* and manifest *Apasmara*.
- Due to this pathological mechanism, the person will be presented with stupor, malfunctioning of mental activities due to involvement of *Rajas* and *Tamas*

- 35 Loud noises or flashing lights are considered to be responsible for occurrence of grand mal epilepsy.
- 36 When an individual is consuming incompatible and contaminated food substances, the immediate response could be the person may develop either vomiting or diarrhoea associated with gastrointestinal symptoms manifestation. This will lead to abrupt nutrient loss, carbohydrates in particular which leads to the excess formation of ketoacidosis and lactic acidosis. This state is called as metabolic acidosis. This may lead to the state of respiratory alkalosis. Respiratory alkalosis is one of the predisposing factor for the development of seizure attacks. S.N. : Respiratory alkalosis is caused by either hypoxia or metabolic acidosis or increased metabolic demand such as fever etc.
- 37 While discussing this point, let us take the example of *Purisha Vega Dharana* (suppression of urge of defecation). When the person frequently suppressing the urge of defecation, that may lead to hard faecal matter (faecolith or scybala). When the person gets the next urge of defaecation, that will lead to the development of mechanism of respiratory alkalosis secondary to overventilation. This respiratory alkalosis may be the triggering factor for the development of seizure attacks in the susceptible individuals.
- 38 These improper method of food intake will lead to poor digestion and absorption from the alimentary canal. When an individual is consuming incompatible and contaminated food substances, the immediate response could be the person may develop either vomiting or diarrhoea associated with gastrointestinal symptoms manifestation. This will lead to abrupt nutrient loss, carbohydrates in particular which leads to the excess formation of ketoacidosis and lactic acidosis. This state is called as metabolic acidosis. This may lead to the state of respiratory alkalosis. Respiratory alkalosis is one of the predisposing factor for the development of seizure attacks. S.N. : Respiratory alkalosis is caused by either hypoxia or metabolic acidosis or increased metabolic demand such as fever etc.
- 39 Sexual intercourse with menstruating woman may be a risk factor the development of seizure attacks. This may happen because of two reasons. Either there may be respiratory alkalosis or there may be appearance of fever in the patient. The persons who are having the history of grand mal epilepsy, this may trigger the episode of seizure attacks.
- 40 All these activities will lead to extreme emotional stimuli which are the triggering factors of grand mal epilepsy.

- Thus, leading to violent movements of the arms, legs, brows, eyes; grinding of the teeth, froth comes out of the mouth, the person will see non-existent things, excessive salivation, falls on the ground with eyes open and regain consciousness after a bouts of convulsant attacks as if from the sleep.

Purvarupa⁴¹

तस्येमानि पूर्वरूपाणि भवन्ति तद्यथा - भ्रूव्यु- दासः सततमक्ष्णोर्वैकृतमशब्दश्रवणं लालासिङ्घाण- प्रस्रवणमनन्नाभिलषण- मरोचकाविपाको हृदयग्रहः कुक्षेराटोपो दौर्बल्यमस्थिभेदोऽङ्गमद मोहस्तमसो दर्शनं मूर्च्छा भ्रमश्चाभीक्षणं स्वप्ने च मदनर्तनव्यध- नव्यथनवेपनपतनादीनीति॥6॥

Ca.Ni. - 8/6

हृत्कम्पः शून्यता खेदो ध्यानं मूर्च्छा प्रमूढता।

निद्रानाशश्च तस्मिंस्तु भविष्यति भवन्त्यथ॥7॥

Su.Ut. - 61/7

- ⊗ **Bhru Vyudasa** – contraction of eye brows
- ⊗ **Satatam Akshorvikriti** – frequent abnormal movement of eyes
- ⊗ **Ashabda Shrivana** – hearing of non-existing sounds
- ⊗ **Lala, Singhanaka Prasavana** – excessive salivary and nasal secretions
- ⊗ **Anannabhilashana** – disinclination to take food
- ⊗ **Arochaka** – tastelessness
- ⊗ **Avipaka** – indigestion
- ⊗ **Hridayagraha** – stiffness in the chest region
- ⊗ **Kukshi Atopa** – distension of the abdomen
- ⊗ **Daurbalya** – weakness
- ⊗ **Asthibheda** – tearing type of pain in the bones
- ⊗ **Angamarda** – bodyache
- ⊗ **Moha** – altered state of consciousness
- ⊗ **Tama Darshana** – darkness in front of the eyes
- ⊗ **Murcha** – unconsciousness
- ⊗ **Bhrama** – giddiness
- ⊗ **Abhiksham Svapne Mardana, Vyadhana, Vyathana, Vepana, Patana etc** – dreams of narcosis, dancing, piercing, aching, shivering and falling

As per Sushruta

- ⊗ **Hritkampa** – tremors in the cardiac region
- ⊗ **Shunyata** – feeling of vacantness
- ⊗ **Sveda** – perspiration
- ⊗ **Dhyana** – unusual thinking
- ⊗ **Murcha** – unconsciousness
- ⊗ **Pramudhata** – fainting or dullness
- ⊗ **Nidranasha** – loss of sleep

Classification of Apasmara

- ⊗ **Sankhya Samprapti: 4**

- *Vataja Apasmara*
- *Pittaja Apasmara*
- *Kaphaja Apasmara*
- *Sannipataja Apasmara*

Vataja Apasmara

वेपमानो दशेदन्तान् श्वसन् फेनं वमन्नपि॥1॥

यो ब्रूयाद्विकृतं सत्त्वं कृष्णं मामनुधावति।

ततो मे चित्तनाशः स्यात्सोऽपस्मारोऽनिलात्मकः॥

Su.Ut. - 61/11-12

तत्रेदमपस्मारविशेषविज्ञानं भवति तद्यथा- अभीक्ष्णम- पस्मरन्तं, क्षणेन संज्ञां प्रतिलभमानम्, उत्पिण्डिताक्षम् असाम्ना विलपन्तम्, उद्धमन्तं फेनम्, अतीवाध्मातग्रीवम्, आविद्धशिरस्कं, विषमविनताङ्गुलिम्, अनवस्थितपाणि- पादम्, अरुणपरुषयावनखनयनवदनत्वचम्, अनवस्थितचपल परुषरूपरूपदर्शिनं, वातलानुपशयं, विपरीतोपरायं च वातेनापस्मरन्तं विद्यात्॥ (1)॥

Ca.Ni. - 8/8

- ⊗ **Vepamana** – tremors
- ⊗ **Dantan Dashet** – biting one's teeth
- ⊗ **Shvasa** – breathlessness
- ⊗ **Bruyat Vikrita Krishna Mamanudhavati** – patient utters black substances are running after him
- ⊗ **Abhiksham Apasmara Kshanena Pratilabhamana Samjna** – frequent convulsions followed by regaining the consciousness instantaneously
- ⊗ **Utpinditaksha** – protruded eyes
- ⊗ **Asamna Vilapantam** – crying recklessly
- ⊗ **Phena Udvamantam** - emitting froth from the mouth
- ⊗ **Ativa Adhmata Griva** – excessive heaviness and rigidity of neck
- ⊗ **Aviddha Shira** – bending of the head to one side
- ⊗ **Vishama Vinatanguli** – irregularly contracted fingers
- ⊗ **Anavasthita Panipada** – instability of hands and feet
- ⊗ **Aruna, Parusha, Shyava Nakha, Nayana, Vadana and Tvak** – reddish, blackish and rough nails, eyes, face and skin
- ⊗ **Anavasthita, Chapala, Parusha, Ruksha Rupadarshina** – frickle, coarse and dry objects
- ⊗ **Vatala Anupashaya and Viparita Upashaya** – aggravation of the symptoms on consuming Vata provoking factors and relief of symptoms on consumption of Vata pacifying factors

G K 41 As per current understanding, this stage is termed as aura. Aura may consists of sensory, autonomic or psychic sensations (e.g. parasthesias, a rising epigastric sensation, abnormal smells, a sensation of fear. For example, excessive salivary and nasal secretions, frequent abnormal movement of the eyes, indigestion, distension of the abdomen etc can be attributable to autonomic stimulation.

Pittaja Apasmara

तृतापखेदमूच्छत धुन्वन्नङ्गानि विह्वलः।

यो ब्रूयाद्विकृतं सत्त्वं पीतं मामनुधावति॥1 3॥

ततो मे चित्तनाशः स्यात्स पित्तभव उच्यते।

Su.Ut. – 61/13-14

अभीक्ष्णमपस्मरन्तं, क्षेणेन संज्ञां प्रतिलभमानम्, अवकृजन्तम्, आस्फालयन्तं भूमिं हरितहा- रिद्रताघ्ननखनयनवदनत्वचं, रुधिरक्षितोग्रभैरवा- दीप्तरूपित रूपदर्शिनं, पित्तलानुपशयं, विपरीतोपशयं च पित्तेनापस्मरन्तं विद्यात्॥ (2)॥

Ca.Ni. – 8/8

- Trit – thirst
- Daha – burning sensation
- Sveda – sweating
- Murcha – unconsciousness
- Dhunvan Angani Vihvala – shaking of body parts
- Bruyat Vikrita Pitam Mamanudhavati – person says yellowish substances running after him
- Abhiksham Apasmarantam Kshanena Samjnam Pratilabhamanam – frequent fits regaining consciousness instantaneously
- Avakujantam – breathing with sounds
- Asphalayantam Bhumi – rubbing the earth
- Harita, Haridra, Tamra, Nakha, Nayana, Vadana, Tvak – greenish, yellowish and coppery discolouration of nails, eyes, face and skin
- Rudhirokshitogra Bhairavadipta Rushita Rupa Darshina – vision of reddish, agitated, fierce, luminous and irritated objects
- Pittala Anupashaya, Viparita Upashaya – signs and symptoms of Apasmara aggravated by the Pitta provoking factors and alleviated by Pitta pacifying factors

Kaphaja Apasmara

शीतहृल्लासनिद्रार्तः पतन् भूमौ वमन् कफम्॥1 4॥

यो ब्रूयाद्विकृतं सत्त्वं शुक्लं मामनुधावति।

ततो मे चित्तनाशः स्यात्सोऽपस्मारः कफात्मकः॥1 5॥ Su.Ut. – 61/14-15

चिरादपस्मरन्तं चिराच्च संज्ञां प्रतिलभमानं, पतन्तम्, अनति विकृतचेष्टं, लालामुद्रमन्तं, शुक्लनखनयनवदनत्वचं, शुक्लगुरुस्निग्धरूपदर्शिनं, ले मलानुपशयं, विपरीतोपशयं च लेप्मणाऽपरम- रन्तं विद्यात्॥ (3)॥

Ca.Ni. – 8/8

- Shita – sense of coldness
- Hrilasa – nausea

- Nidra – sleep
- Patan Bhumau - Vaman Kapham – vomits mucus (Kapha) while falling on the ground
- Bruyat Vikrita Shuklam Mamanudhavati – patient says whitish substances are running after him
- Chirat Apasmaranta, Chirat Samjnam Pratilabhamana – person will have convulsions which are usually slow in the movement and he will have delayed recovery from unconsciousness
- Patanta – frequently falling down
- Anativikrita Cheshta – absence of much distorted activities
- Lalam Udvamanta – dribbling of saliva
- Shukla Nakha, Nayana, Vadana, Tvak – whitish discolouration of nails, eyes, face and skin
- Shukla, Guru, Snigdha, Rupa Darshina – vision of white, heavy and unctuous objects
- Shleshma Anupashaya, Viparita Upashaya – worsening of the signs and symptoms on exposure to Shleshma provoking factors and alleviation on exposure to Shleshma pacifying factors

Sannipataja Apasmara

सर्वलिङ्गसमवायः सर्वदोषप्रकोपजे।

Su.Ut. – 61/17

समवेतसर्वलिङ्गमपस्मारं सान्निपातिकं विद्यात्, तमसाध्यमाचक्षते॥ (4)॥

Ca.Ni. – 8/8

In Sannipataja Apasmara, mixed signs and symptoms mentioned in earlier Dosha dominant Apasmara are observed.

Sadhyasadhya of Apasmara

अपस्मारः स चासाध्यो यः क्षीणस्यानवश्च यः॥1 2॥

पक्षाद्वा द्वादशाहद्वा मासाद्वा कुपिता मलः।

अपस्माराय कुर्वन्ति वेगं किञ्चिदथान्तरम्॥1 3॥ Ca.Ci. – 10/12-13

In following conditions, Apasmara is Asadhya (incurable) in nature

- Kshina⁴² – If the person suffering from Apasmara is very emaciated
- Anava⁴³ – If the duration of Apasmara is of longer duration
- Pakshat, Dvadashahat and Masat Vegam Kurvanti⁴⁴ – If the person is getting episode of Apasmara once in 12 days, 15 days or a month even though the episode persist for shorter duration

- G K
- 42 When an individual suffers from Apasmara (epilepsy), there will be tonic and clonic seizures in parts of the body or in case of grand mal epilepsy, the symptoms are seen all over the body. This event is responsible for large amount of nutrient utilisation and large amount of heat is generated which further leads to the rapid depletion of nutrients including glucose and electrolytes. If the individual is emaciated, there is already depletion of the nutrients in the body, if he is suffering from grand mal epilepsy, there is no nutrients to supplement the depleted energy from the body. That may be the reason, we can say such type of disease is considered as Asadhya.
 - 43 If the person is suffering from Apasmara which is of longer duration, it may either indicate the severity of the disease or it may suggest rapid nutrient depletion from the body or it may imply the extent of the lesion depending upon the appearance of the symptoms in the part of whole of the body.
 - 44 The recurrence of the symptoms may indicate the Vyabichari Hetutva (dormancy of the morbid Dosha present inside the human body) of the Nidana in the disease manifestation. This indicates rapid nutrient loss from the body depending upon the part of the body afflicted.

Key summary of the chapter

Unmada

स्मृतेरपगमं प्राहुरपस्मारं भिषग्विदः। तमःप्रवेशं बीभत्सचेष्टं धीसत्त्वसंप्लवात्॥३॥

Ca.Ci. – 10/3

- When aggravated Dosha takes upward movement and take shelter in Manovaha Srotas to cause disturbance in the Manas. That is the reason, this disease is termed as Unmada.

Pratyatma Lakshana of Unmada

उन्मादं पुनर्मनोबुद्धिसंज्ञाज्ञानस्मृतिभक्तिशीलचेष्टाचारविभ्रमं विद्यात्॥५॥

Ca.Ni. – 7/5

- Manas, Buddhi, Sanjnanana, Smriti, Bhakti, Shila, Cheshta and Achara Vibhrama (deviations in the pattern of person's thinking, feeling, action in above said parameters) are the hallmark of the disease Unmada.

Samanya Lakshana of Unmada

धीविभ्रमः सत्त्वपरिप्लवश्चपर्याकुला दृष्टिरधीरता च। अबद्धवाक्त्वं हृदयं च शून्यं सामान्यमुन्मादगदस्य लिङ्गम्॥६॥

Ca.Ci. – 9/6

- Dhi Vibhrama (improper understanding), Satva Pariplava (instability of the mind), Paryakula Drishti (unsteady gaze), Adhirata (feeling of fear), Abaddha Vaktva (irrelevant and incomprehensible speech) and Hridaya Shunyata (emptiness of the heart or mind) are the common Lakshana mentioned in the context of Unmada as per Charaka.
- As far as Sankhya Samprapti of Unmada is concerned, it is of 5 types – Vataja, Pittaja, Kaphaja, Sannipataja and Agantuja or Bhutaja Unmada.
- Himsa (causing harm to either self or others), Rati (excess desire) and Abhyarchana (worshipping) are three main reactions seen in the patients of Agantuja Unmada.

Apasmara

स्मृतेरपगमं प्राहुरपस्मारं भिषग्विदः। तमःप्रवेशं बीभत्सचेष्टं धीसत्त्वसंप्लवात्॥३॥

Ca.Ci. – 10/3

- Apasmara is characterised by the clinical features of Apageama of Smriti (loss of memory), Tama Pravesha (darkness in front of the eyes or loss of consciousness) associated with Bhibhatsa Cheshta (involuntary movements of the body). As far the Dosha involvement is concerned, this is manifested due to the vitiation of Sharirika Dosha along with Tamo Dosha involvement. The clinical features of this spectrum of the diseases will mimic the features of seizure disorders.
- Sankhya Samprapti of Apasmara is of 4 types: 3 types of Doshaja Apasmara and one type of Sannipataja Apasmara.
- If an individual is suffering from Apasmara and he is having extreme emaciation and if the duration of the illness is prolonged and if the disease recurs for every 12 days or 15 days or 30 days, such type of Apasmara is considered as Asadhya in nature.

MCQs

1. Sankhya Samprapti of Unmada as per Charaka is

- a. 4; b. 5;
- c. 6; d. 7

Answer: (b)

2. Paryakula Drishti is the specific Lakshana seen in

- a. Apasmara;
- b. Unmada;
- c. Atatvabhinivesha;
- d. Apatantraka

Answer: (b)

3. Rahaskamata is the Lakshana of

- a. Vataja Unmada;
- b. Kaphaja Unmada;
- c. Pittaja Unmada;

d. Kaphaja Apasmara

Answer: (b)

4. Svapne Kalusha Bhojana is the Purvarupa observed in

- a. Apasmara;
- b. Atatvabhinivesha;
- c. Unmada;
- d. Arochaka

Answer: (c)

5. As per Sushruta, the types of Unmada are

- a. 5; b. 6;
- c. 4; d. 8

Answer: (b)

6. Vishaja Unmada is the contribution of

- a. Charaka; b. Yoga Ratnakara;
- c. Sushruta; d. Madhavakara

Answer: (c)

7. Vigata Unmada Lakshana is/ are

- a. Buddhi Prasada; b. Mana Prasada;
c. Prakritistha Dhatu; d. All of the above

Answer: (d)

8. Rati type of reflection is seen in which of the following type of Agantuja Unmada?

- a. Deva; b. Pitrija;
c. Pischacha; d. Gandharva

Answer: (d)

9. Himsa type of reflection is seen indominant Agantuja Unmada?

- a. Satva; b. Rajo Dosha dominant;
c. Tamo Dosha dominant d. Both c and d

Answer: (c)

10. Dhi Vibhrama isof Unmada.

- a. Nidana; b. Purvarupa;
c. Pratyatma Lakshana; d. Upadrava

Answer: (c)

11. Late returning of the sensation is the symptom of Apasmara.

- a. Vataja; b. Pittaja;
c. Kaphaja; d. Sannipataja

Answer: (c)

12. Smriterapadhvamsah is the Lakshana of

- a. Apasmara; b. Unmada;
c. Bhrama; d. Atatvabhinivesha

Answer: (a)

13. Hrit Kampa Shunyata, Sveda, Dhyana, Murcha, Pramudhata and Nidranasha are the Purvarupa of

- a. Sanyasa; b. Apasmara;
c. Unmada; d. Atatvabhinivesha

Answer: (b)

14. Which of the following is not the type of Apasmara?

- a. Vishaja Apasmara; b. Raktaja Apasmara;
c. Madyaja Apasmara; d. All of the above

Answer: (d)

15. Olfactory hallucinations are seen in

- a. Delirium;
b. Ergot poisoning;
c. Temporal lobe epilepsy;
d. Schizophrenia

Answer: (a)

16. As per Charaka, the types of Apasmara mentioned are

- a. 3; b. 4;
c. 5; d. 7

Answer: (b)

17. Visual hallucinations are seen in

- a. Delirium; b. Cocaine poisoning;

- c. Schizophrenia; d. None of the above

Answer: (a)

18. Which of the following Dosha is dominant in the patient of Apasmara?

- a. Satva; b. Rajas;
c. Tamas; d. All of the above

Answer: (c)

19. A patient of Apasmara gets an episode of Bhibhatsa Cheshta following anxiety. In this example, anxiety is termed as

- a. Sannikrishta Hetu; b. Viprakrishta Hetu;
c. Vyabhichari Hetu; d. Pradhanika Hetu

Answer: (c)

20. Auditory hallucinations are common in

- a. Schizophrenia; b. Ergo poison;
c. Dhatura poison; d. Temporal lobe epilepsy

Answer: (a)

Chapter resources

- ⊛ Charaka Samhita Nidana Sthana 7th chapter – Unmada Nidana
- ⊛ Charaka Samhita Chikitsa Sthana – 9th chapter – Unmada Chikitsa Adhyaya
- ⊛ Sushruta Samhita Uttara Tantra – 62nd chapter – Unmada Pratishedha Adhyaya
- ⊛ Charaka Samhita, Nidana Sthana 8th chapter – Apasmara Nidana Adhyaya
- ⊛ Charaka Samhita, Chikitsa Sthana 10th chapter – Apasmara Chikitsa Adhyaya
- ⊛ Sushruta Samhita, Uttara Tantra 61st chapter – Apasmara Pratishedha Adhyaya
- ⊛ Madhava Nidana – 21st chapter – Apasmara Nidana Adhyaya
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